Atezolizumab + bevacizumab for hepatocellular carcinoma (HCC)

Treatment scope · NCCP 00831 This is the HCC combination, not atezolizumab alone or a lung-cancer chemotherapy combination. Existing liver disease and bleeding risk need review alongside possible immune side effects. Your team assesses each medicine separately.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Atezolizumab intravenous (IV)/subcutaneous (SC) + bevacizumab IV

Active medicines: atezolizumab + bevacizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis; individualise rescue treatment under NCIS minimal-risk/local policy.
- No routine premedication specified; assess prior administration reaction.
- Carry immunotherapy alert card and oncology contact plan.
- Manage varices, blood pressure (BP), urine-protein findings and procedure timing.
- Do not mask new significant diarrhoea with self-treatment; assess immune colitis and other causes.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00831 v2a · p4, 10–12 · NCIS Medical Oncology antiemetics V8: minimal-risk row

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required